

created an early disclosure framework but, in doing so, it did not repeal existing statutory protections or require manufacturers to publicly disseminate competitively sensitive internal documents.

Had the Legislature intended to strip manufacturers of all confidentiality protections for documents disclosed under section 871.26(h)(12) and (15), it would have said so expressly, particularly given the longstanding practice of protective orders in commercial litigation and the Legislature's awareness that these manuals and policies contain competitively sensitive information. The statute imposes disclosure, not unprotected public dissemination.

Kia does not seek to avoid production. It seeks to produce under the safeguard of a protective order. Kia has represented that it is willing to produce these documents but pursuant to an appropriate protective order. That approach can be reconciled with the statute's disclosure mandate while preserving the confidentiality protections that coexist with it. On the present record, Kia has made a sufficient showing of good cause, and entry of a protective order is appropriate. Accordingly, the motion is **GRANTED**.

Summary: GRANT Defendant Kia America Inc.'s Motion for Protective Order. Moving party to submit a protective order for the Court's signature.

6.

CASE #	CASE NAME	HEARING NAME
CVRI2602141	SHAWANDA R. OBEY, MD, MPH vs MAGELLA MEDICAL GROUP, INC.	ANTI-SLAPP MOTION (SPECIAL MOTION TO STRIKE)

Tentative Ruling:

Moving party: Defendant San Antonio Regional Hospital
Represented by: Cole Pedroza LLP

Responding party: Plaintiff Shawanda R. Obey, MD, MPH
Represented by: pro per

Plaintiff Shawanda R. Obey, M.D. ("Plaintiff") is an obstetrician-gynecologist ("OB/GYN"). She joined the medical staff at Riverside Health System, L.P. dba Riverside Community Hospital ("RCH") in 2002 and was employed by Pediatrix Medical Group, Inc. ("Pediatrix"). She contends that after she joined RCH, she received a telephone call about lawsuits she had filed against other hospitals and medical groups and was told that there were concerns about her being at RCH because of these lawsuits. Shortly after she started work, she complained about a toxic hospital culture. She contends that false and discriminatory statements that were harmful to her career were made while she was at RCH. She reported unsafe practices and conditions at RCH and was retaliated against while working at RCH. She was allegedly fired by Pediatrix and opened her own practice, while still retaining privileges at RCH and two other hospitals.

She contends that when her privileges were up for renewal at Pomona Valley Hospital Medical Center ("PVHMC"), it wanted information from RCH. RCH refused to provide this information without Plaintiff executing an "Authorization for Release of Information." Plaintiff contends that she refused to sign this document because it was more than a standard credential release. Due to this, her credentials were not renewed at PVHMC. She contends that her credentials were wrongfully suspended by San Antonio Regional Hospital ("SARH"). She informed RCH of this information. However, RCH did nothing with this information until Plaintiff's credentials were up for renewal. During renewal, it asked for information from SARH and Plaintiff needed to execute a "special release" from SARH to provide the necessary information for RCH to complete the credential process. Plaintiff refused to execute this form because she disagreed with the scope of the waiver. Due to this, RCH only extended her hospital credentials for a few months and she asserts this limited extension was improper.

On April 14, 2026, Plaintiff filed this action asserting the following causes of action: (1) whistleblower retaliation, (2) wrongful termination in violation of public policy, (3) intentional interference with prospective economic advantage, (4) negligent interference with prospective economic advantage, (5) civil conspiracy, (6) denial of fair procedure, (7) unfair competition, (8) breach of implied covenant of good faith and air dealing, (9) wage and hour violations, and (10) intentional infliction of emotional distress.

SARH now moves the Court for an order striking the 1st, 3rd, 4th, 5th, 6th, 7th and 10th causes of action (only claims asserted against it). (C.C.P., § 425.16.) It argues all causes of action asserted arise from acts and conduct in furtherance of its rights under both the United States and California Constitutions and that the acts and conduct is protected by C.C.P., § 425.16(b), (e)(2), and (e)(4). As to Prong One, SARH identifies three protected-activity buckets: (1) SARH's medical staff sought to have Dr. Obey execute an authorization and release form for purposes of providing peer review information to RCH, which SARH argues "constitutes communications made to facilitate a peer review proceeding, which is an 'official proceeding'" under *Bonni v. St. Joseph Health System* (2021) 11 Cal.5th 995; (2) reports to the Medical Board and to the National Practitioner Data Bank "qualify as 'statement[s] or writing[s] made in connection with an issue under consideration' in an 'official proceeding,'" and (3) the alleged withholding of information in connection with challenge to an adverse reaction in peer review is protected right not to speak. On Prong Two, SARH argues the Lab. Code § 1102.5 slice fails because Plaintiff does not allege that she was an "employee" of SARH; in fact, she was not an employee of SARH at any relevant time; that the release-form demand is statutorily authorized by Business & Professions Code § 809.08 and therefore was not independently wrongful by any recognized legal standard; that the Business & Professions Code § 805 and NPDB reports carry statutory immunity; and that the negligent-interference and fair-procedure claims fail because the Judicial Review Committee hearing has not yet concluded Plaintiff has not sought a hearing-officer ruling on her document requests; that the sixth cause of action is nonviable; and the UCL and IIED counts fail for lack of unlawful/unfair conduct and lack of outrageous conduct. SARH argues Plaintiff is unable to demonstrate a probability of prevailing on

the merits of these causes of action. It reserves the right to move for attorney fees. (C.C.P., § 425.16(c) and California Rules of Court, rule 3.1702.)

In opposition, Plaintiff argues this is a medical-staff whistleblower-retaliation action, not a challenge to legitimate peer-review speech. SARH restricted Plaintiff's obstetrical privileges on July 3, 2025 after years of documented patient-safety reporting to the hospital, CDPH, Cal/OSHA, the licensing boards, and the Joint Commission. Plaintiff argues that the Court must engage in act-by-act analysis, so the restriction, its continuation, and resulting exclusion are adverse decisions, not protected merely because peer review preceded them, the § 805/NPDB reports and peer-review statements are offered only to prove notice, motive, pretext, or causation, not as standalone torts, so any portion resting exclusively on protected reporting should be isolated rather than used to strike seven mixed claims wholesale. (*Bonni v. St. Joseph Health System* (2021) 11 Cal.5th 995.) On Prong Two, Plaintiff contends she establishes minimal merit under Health and Safety Code section 1278.5 through protected reports, SARH's knowledge, unfavorable privilege changes, close timing, the 120-day presumption, competence evidence, and pretext, that Plaintiff does not rely on Labor Code section 1102.5 against SARH because SARH did not employ her, and the pending § 809 hearing does not bar the claim.

In reply, SARH argues Plaintiff reimagined her allegations in opposition to escape anti-SLAPP scrutiny, but the Court must review the allegations as stated in the complaint. SARH re-argues the points presented in its moving papers.

Analysis

I. Anti-SLAPP Standard

"The Legislature enacted...anti-SLAPP statute [strategic lawsuit against public participation]...to provide a procedural remedy to dispose of lawsuits that are brought to chill the valid exercise of constitutional rights. [Citation.]" (*Rusheen v. Cohen* (2006) 37 Cal.4th 1048, 1055.) C.C.P. § 425.16 provides a two-Prong test in determining whether a cause of action constitutes a SLAPP. (*Navellier v. Sletten* (2002) 29 Cal.4th 82, 88; C.C.P., § 425.16(b)(1).) The moving party bears the initial burden of showing that the action falls within the class of suits subject to the special motion to strike. (*Wilcox v. Sup. Ct.* (1994) 17 Cal.App.4th 809, 819.) "In the anti-SLAPP context, the critical point is whether the plaintiff's cause of action itself was based on an act in furtherance of the defendant's right of petition or free speech." (*City of Cotati v. Cashman* (2002) 29 Cal.4th 69, 78.) Once the defendant makes such a prima facie showing, the burden shifts to the plaintiff to establish a "probability" that it will prevail on whatever claims are asserted against the moving defendant. (C.C.P., § 425.16(b).) The plaintiff must demonstrate that the complaint is both legally sufficient and supported by a sufficient prima facie showing of facts to sustain a favorable judgment. (*Navellier, supra*, 29 Cal.4th at 89, 93.)

Litigation of an anti-SLAPP motion involves a two-Prong process. (*Bonni v. St. Joseph Health Sys.* (2021) 11 Cal.5th 995, 1009.) First, "the moving defendant bears

the burden of establishing that the challenged allegations or claims ‘aris[e] from’ protected activity in which the defendant has engaged.” (*Id.*) Second, for each claim that does arise from protected activity, the plaintiff must show the claim has “at least minimal merit.” (*Id.*) If the plaintiff cannot make this showing, the court will strike the claim. (*Id.*)

“At this first Prong, courts are to “consider the elements of the challenged claim and what actions by the defendant supply those elements and consequently form the basis for liability.” [citation.] The defendant’s burden is to identify what acts each challenged claim rests on and to show how those acts are protected under a statutorily defined category of protected activity. [citation.]” (*Id.*) The anti-SLAPP statute identifies four categories of protected activity. (§ 425.16, subd. (e)(1)–(4).) Here, SARH invokes two of them. Section 425.16, subdivision (e)(2) protects “any written or oral statement or writing made in connection with an issue under consideration or review by a legislative, executive, or judicial body, or any other official proceeding authorized by law.” Subdivision (e)(4) covers “any other conduct in furtherance of the exercise of the constitutional right of petition or the constitutional right of free speech in connection with a public issue or an issue of public interest.”

When a claim is based on both protected and unprotected activity, the moving defendant bears the burden of identifying all allegations of protected activity, and the claims for relief supported by them. (*Baral v. Schnitt* (2016) 1 Cal. 5th 376, 382.). When relief is sought based on allegations of both protected and unprotected activity, the unprotected activity is disregarded at this stage. (*Id.* at 396.) If the court determines that relief is sought based on allegations arising from activity protected by the statute, the second Prong is reached. (*Ibid.*)

At the second Prong, the burden shifts to the plaintiff to demonstrate that each challenged claim based on protected activity is legally sufficient and factually substantiated. (*Id.*) The court, without resolving evidentiary conflicts, must determine whether the plaintiff’s showing, if accepted by the trier of fact, would be sufficient to sustain a favorable judgment. (*Id.*) If not, the claim is stricken. (*Id.*)

II. Merits

Medical peer review proceedings constitute official proceeding[s] authorized by law within the meaning C.C.P., § 425.16(e)(2). (*Kibler v. Northern Inyo County Local Hospital Dist.* (2006) 39 Cal.4th 192, 199.) Hospital peer review proceedings constitute official proceedings authorized by law within the meaning of section 425.16, subdivision (e)(2) because a hospital must report to the Medical Board of California (Medical Board), which licenses physicians, any hospital action that restricts or revokes a physician’s staff privileges as a result of a determination by a peer review body. (*Id.* at 200.) Communications made in connection with such proceedings are protected and may be struck only if the speech or petitioning activity itself is the wrong complained of, and not just evidence of liability or a Prong leading to some different act for which liability is asserted. (*Bonni v. St. Joseph Health Sys.* (2021) 11 Cal.5th 995 (2021) [distinguished between protected peer review communications, including statements during proceedings and mandatory reports to licensing bodies, and unprotected final discipline decisions imposed after exhausted review], citing to *Park v. Board of Trustees of California State University* (2017) 2 Cal.5th 1057, 1060.)

First Cause of Action – Whistleblower Retaliation (§ 1278.5 / § 1102.5)

Health and Safety Code section 1278.5 forbids health facilities from discriminating or retaliating against certain individuals, including medical staff members, for presenting complaints concerning the quality of patient care to other members of the medical staff, the facility, or other responsible entities. (Health & Saf. Code, § 1278.5, subd. (b)(1).) A claim under this statute requires proof of discriminatory treatment, which may be shown by “any unfavorable changes in” a medical staff member’s “contract, employment, or privileges ... or the threat of” such changes. (*Id.*, subd. (d)(2).)

Here, Plaintiff alleges SARH retaliated against her for making protected safety complaints by restricting her obstetrical privileges on July 3, 2025, behavioral-agreement coercion, the maintained release condition, § 805/NPDB reporting, and alleged § 809.2(d) withholding of information from RCH.

Prong One: SARH meets its burden. The allegations rest on four categories of protected activity:

1. Section 805 and NPDB Reports: Plaintiff alleges the Hospitals retaliated against her by reporting her summary suspensions to the Medical Board of California and National Practitioner Data Bank, conduct *Bonni* held protected as written statements to the state’s licensing agency concerning restrictions imposed by a peer review body.
2. July 3, 2025 Summary Restriction: The restriction initiated formal peer review proceedings, functioning as the investigative measure triggering statutory reporting obligations and hearing rights under Business & Professions Code section 809. Unlike the post-hearing termination decisions in *Bonni*, After a lengthy series of evidentiary hearings, the hearing committee determined that the medical executive committee had sustained its burden on three of 18 charges against *Bonni* but had not shown by a preponderance of the evidence that either the summary suspension or the termination recommendation was warranted, the July 3 restriction was preliminary peer review action, not final discipline after exhausted review. The restriction is conduct in connection with SARH’s official peer review proceeding.
3. Communications Regarding RCH Information-Sharing: Written demands for a release form, rejection of plaintiff’s narrower authorization, and communications conditioning disclosure are statements made in connection with two official proceedings: SARH’s ongoing peer review and RCH’s credentialing proceeding (itself peer review under *Kibler*). Business & Professions Code section 809.08(d) authorizes conditioning disclosure on receiving a release. SARH’s invocation of this statutory framework through written communications constitutes protected activity.
4. MEC Investigation and Committee Actions: The Medical Executive Committee’s evaluation, preliminary determinations, and ongoing review through May 2026 reappointment proceedings are peer review committee actions within the official proceeding. These are analogous to the medical

executive committee's arguments before the peer review panel that the suspension should be upheld, and criticizing Dr. Bonni's professional abilities in the course of peer review proceedings, which *Bonni* held protected.

Prong Two: Civil Code section 47(b) provides absolute immunity for any written or oral statement or writing made in connection with an issue under consideration or review by a legislative, executive, or judicial body, or any other official proceeding authorized by law. (The litigation privilege bars tort claims based on peer review communications even when allegedly motivated by retaliation. Additionally, Business & Professions Code sections 809.05 and 809.6 immunize peer review participants absent malice.

The allegations seeking damages solely on **report content** are stricken under *Baral*, but the reports remain admissible as pretext evidence. Plaintiff establishes minimal merit on the § 1278.5 core: protected reporting; restriction within 48 hours of the July 1 complaint, triggering the 120-day presumption; no documented clinical justification; pretextual psychiatric evaluation without mental-health history; favorable findings ignored; and maintenance of the restriction to reach the 30-day reporting threshold. If release-form correspondence is treated as protected, minimal merit is still shown: SARH demanded blanket authorization, mental-health disclosure, and release of known and unknown claims exceeding § 809.08(d) and rejected Plaintiff's compliant substitute without curative dialogue causing the 81-day delay and 120-day RCH renewal.

Accordingly, the Court grants the motion in part (as to report content) as to the 1st cause of action.

Third Cause of Action – Intentional Interference with Prospective Economic Advantage

The elements of intentional interference with prospective economic advantage are: (1) economic relationship between plaintiff and third party with the probability of future economic benefit to the plaintiff; (2) defendant's knowledge of the relationship; (3) defendant's intentional acts designed to disrupt the relationship; (4) actual disruption of the relationship; and (5) economic harm to the plaintiff. (See *Sole Energy Co. v. Petrominerals Corp.* (2005) 128 Cal.App.4th 212, 241.) Further, to establish intentional interference with prospective economic advantage, a plaintiff must plead and prove that the defendant's interference was wrongful by some legal measure other than the fact of interference itself (i.e., that the defendant engage in independently wrongful act beyond disruption of the economic relationship). (*Ibid.*)

Prong One. Dr. Obey alleges SARH disrupted her RCH privileges through the restriction, its maintenance, the overbroad release condition, the 81-day delay, and the resulting 120-day RCH renewal. As discussed above, the noncommunicative restriction and its continuation do not arise from protected activity. The § 805/NPDB reports, peer-review statements to RCH, and any communications with the Joint Commission

constitute protected statements in connection with § 809 official proceedings. SARH meets Prong One as to those slices.

Prong Two. Plaintiff cannot establish wrongful interference. RCH's credentialing decision was made independently by RCH's peer review body based on its own evaluation. SARH's conditioning of disclosure on a statutory release form authorized by section 809.08(d) was lawful conduct. Plaintiff's refusal to provide the requested authorization broke any causal chain. Plaintiff alleges no false or misleading statements by SARH to RCH. Mere delay in responding, standing alone, does not constitute intentional interference. The claim is additionally barred by litigation privilege and common interest privilege (Civil Code § 47(c)) for communications between peer review bodies sharing information pursuant to statutory authorization. **The Court grants the motion as to this cause of action.**

Fourth Cause of Action – Negligent Interference with Prospective Economic Advantage

“The tort of negligent interference with prospective economic advantage is established where a plaintiff demonstrates that (1) an economic relationship existed between the plaintiff and a third party which contained a reasonably probable future economic benefit or advantage to plaintiff; (2) the defendant knew of the existence of the relationship and was aware or should have been aware that if it did not act with due care its actions would interfere with this relationship and cause plaintiff to lose in whole or in part the probable future economic benefit or advantage of the relationship; (3) the defendant was negligent; and (4) such negligence caused damage to plaintiff in that the relationship was actually interfered with or disrupted and plaintiff lost in whole or in part the economic benefits or advantage reasonably expected from the relationship.” (*Venhaus v. Shultz* (2007) 155 Cal.App.4th 1072, 1078.)

Prong One. Plaintiff alleges negligent interference through failure to follow fair procedures, including § 809.2(d) discovery withholding, the restriction, and continuation. The noncommunicative restriction and alleged § 809.2(d) omission do not arise from protected activity. Any peer-review statements or § 805/NPDB reports supplied to third parties as causation are protected. Prong One is carried as to those protected slices.

Prong Two. Plaintiff cannot establish minimal merit. Negligent interference requires breach of an independent duty beyond the interference itself. SARH owed no duty to plaintiff to facilitate her RCH credentialing or to respond on plaintiff's preferred timeline. **The claim is barred by litigation privilege for the same reasons.**

Fifth Cause of Action – Civil Conspiracy

“The elements of an action for civil conspiracy are (1) formation and operation of the conspiracy, and (2) damage to plaintiff resulting from an act or acts done in furtherance of the common design.” (*Klistoff v. Superior Court* (2007) 157 Cal.App.4th 469, 479.) “Civil conspiracy is not an independent tort.” (*Favila v. Katten Muchin Rosenman LLP* (2010) 188 Cal.App.4th 189, 206.)

Prong One. Conspiracy is derivative. The alleged overt acts—the restriction, statutory reports, and communications with RCH—are protected peer review conduct. Conspiracy to engage in protected activity is itself protected.

Prong Two. Without viable predicate torts, the conspiracy claim fails. **The underlying claims are barred by litigation privilege and statutory immunities.**

Sixth Cause of Action – Denial of Fair Procedure

As against SARH, Dr Obey alleges that SARH “denied Plaintiff fair procedure by imposing a summary restriction limited to obstetrical privileges without providing adequate notice, evidentiary support, or a timely and meaningful opportunity to be heard.” (Complaint, ¶ 663.) She further alleges that SARH “failed to provide unredacted materials, including the identity of complainants, complete underlying documentation, and information necessary for Plaintiff to meaningfully respond to the allegations used to justify the restriction.” (Complaint, ¶ 665.) She further alleges that SARH “further conditioned the continuation or restoration of Plaintiff’s privileges on the execution of improper and coercive requirements, including overbroad waivers and referrals for evaluation without adequate justification.” (Complaint, ¶ 666.)

Notably, “Denial of Fair Procedure” claim is no longer a recognized cause of action under California law. (*Asiryan v. Glendale Adventist Medical Center* (2024) 100 Cal.App.5th 947, 966-969.)

Prong One. Dr. Obey alleges deprivation of fair procedure through the restriction and § 809.2(d) withholding, which inherently challenges statements or decisions rendered in the § 809 official proceeding. This claim challenges peer review procedures and rests on peer review committee decisions about document disclosure made during the official proceeding. The committee’s determinations regarding what information to disclose and when constitute peer review deliberations and decisions within the official proceeding, protected under section 425.16(e)(2). SARH meets Prong One.

Prong Two. Section 809.2(d) is a procedural statute that does not create a private right of action. Plaintiff’s remedy for alleged peer review procedural violations is administrative mandate review of the peer review decision, not a standalone tort claim. Moreover, this is not a recognized cause of action. **Accordingly, the motion is granted.**

Seventh Cause of Action – Unfair Competition Law (Bus. & Prof. Code § 17200)

Business & Professions Code § 17200 prohibits any business act or practice that is unlawful, unfair, or fraudulent. A cause of action for violating this statute “borrows” actionable conduct and makes it independently actionable under the unfair competition law. (*Smith v. State Farm* (2001) 93 Cal.App.4th 700, 718.) Plaintiff’s unfair competition claim is predicated on SARH’s insistence that she execute the authorization and release form, which she characterizes as both “unlawful” and “unfair.”

Prong One. The UCL claim incorporates the alleged Health & Safety Code section 1278.5 retaliation and the release form conditioning as predicate unlawful acts. Both rest on protected peer review activity as analyzed above.

Prong Two. Without viable predicate violations, the underlying retaliation and interference claims are barred by privilege and immunity, the UCL claim fails. **SARH's conduct was authorized by statute (sections 809, 809.08, 809.2) and protected by litigation privilege.**

Tenth Cause of Action – Intentional Infliction of Emotional Distress

“A cause of action for intentional infliction of emotional distress exists when there is (1) extreme and outrageous conduct by the defendant with the intention of causing, or reckless disregard of the probability of causing, emotional distress; (2) the plaintiff's suffering severe or extreme emotional distress; and (3) actual and proximate causation of the emotional distress by the defendant's outrageous conduct. A defendant's conduct is outrageous when it is so extreme as to exceed all bounds of that usually tolerated in a civilized community. And the defendant's conduct must be intended to inflict injury or engaged in with the realization that injury will result. Liability for intentional infliction of emotional distress does not extend to mere insults, indignities, threats, annoyances, petty oppressions, or other trivialities.” (*Hughes v. Pair* (2009) 46 Cal.4th 1035, 1050-51.)

Prong One. Dr. Obey alleges IIED from the restriction, psychiatric-evaluation demand, continued exclusion despite favorable findings, and the resulting physical and family consequences. SARH meets its burden. The IIED claim rests on the same protected peer review conduct, the restriction, proceedings, communications, and reports. Those acts are protected.

Prong Two Plaintiff cannot establish outrageous conduct as a matter of law. Conducting statutorily mandated peer review proceedings, imposing preliminary restrictions pending investigation, and invoking statutory protections for information-sharing do not exceed all bounds usually tolerated in a civilized community. The claim is additionally barred by litigation privilege.

The motion is Granted.